CareJiyo

User manual

Business processes, screens and everyday tasks

ENGLISH

A practical starting point for new staff. Understand the whole journey, then use the task guides and videos for practice.

8 November 2026

carejiox.com

CareJiyo Learning

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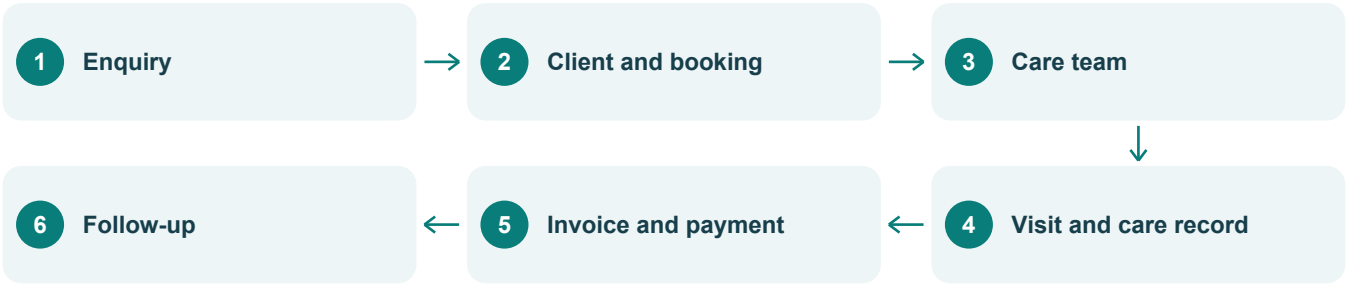
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CHAPTER 01

Start here: how the clinic works

The normal process



In this chapter

What CareJiyo helps you do

Sign in and understand your access

Lists, forms, buttons and saved changes

Know when your work is finished

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

1.1 What CareJiyo helps you do

CareJiyo brings the office and visiting care team into one system. CRM handles enquiries. Operations plans visits and staff. Clinical holds care information. Finance follows invoices and money received. The mobile app helps the assigned nurse or doctor work at the visit. Learn provides instructions and videos.

A contact is someone who has enquired. A client is the person receiving care. A booking is one appointment. An assignment connects a staff member to that booking. A clinical note records the care given. An invoice records the charge; a payment records money received. These are linked records, not interchangeable names for the same thing.

Example: a daughter calls for her father. Record the daughter as the caller or representative and the father as the client receiving care. Create the father's booking, assign the care team, record the visit, and check the resulting invoice. Keep the relationship clear so the right person is contacted and the right person receives care.

Read chapters 1-2 first. Office staff then read bookings and scheduling; clinical staff read mobile and clinical records; finance staff read billing. Use the detailed task videos when you are ready to practise.

1.2 Sign in and understand your access

1. Open https://carejiox.com in Chrome. Use the individual login and password provided to you. The product is called CareJiyo; the current address remains carejiox.com.
2. Check your name, language and clinic. Where available, choose English or Vietnamese using the language control. Check that dates and visit times use the clinic's timezone.
3. Use Home to return to your starting page. Choose an area such as CRM, Operations, Clinical, Finance or Learn. Tabs inside the area open its screens.
4. Check the Catchment Area filter before searching. A catchment is the geographic area your clinic serves. Operations Managers are restricted to their assigned catchments; changing a filter does not grant access to another area. Nurses and doctors also work within their permitted scope and assignments.
5. If a menu or record is missing, check the date, search filter, catchment and assignment. Ask your manager to review access if it still cannot be found. Do not use a colleague's login.

Some screens show the clinic's own name or logo. The same task can have a slightly different layout according to role, language and enabled features. Screenshots illustrate the controls; use the record details and status shown in your own session.

1.3 Lists, forms, buttons and saved changes

Lists show many records; forms show one record. A search narrows the list. Filters select a group, such as today's bookings. Group By organises records under headings. Removing a filter makes more permitted records visible; it does not change the records.

Control	What it means
New / Create	Open a blank record. Check for an existing record first.
Save / Save manually	Store the entered changes. Read any validation message.
Discard all changes	Abandon unsaved edits; it does not reverse an earlier saved change.
Search / Filters	Find records by reference, person, date or status.
List / Calendar / Graph / Pivot	Different ways to display the same permitted data.
Chatter: Log note	Add an internal comment to the record history.
Chatter: Send message	Send a message to the selected recipients; review them first.
Activities	Set a follow-up owner and due date.
Refresh	Reload the displayed information.

A star or highlighted field may indicate required information. Read the field label and help text. Select an existing dropdown value carefully; similarly named clients are not necessarily the same person. Before saving, check the client, reference, date and amount. Reopen important records to confirm the change was stored.

1.4 Know when your work is finished

Each team hands a clear result to the next team. CRM hands over a correct client and request. Operations hands over an agreed booking, service details and accepted staff assignment. The clinician hands over a completed visit and accurate care record. Finance checks the invoice, records actual payments and follows the remaining balance.

A booking marked Completed does not by itself prove that its invoice is paid. A payment marked collected does not necessarily mean field cash has reached the office. A saved note is not necessarily a signed final note. Always check the separate status for the work you are responsible for.

At the end of your shift, review unfinished activities, unassigned visits, incomplete notes and payments awaiting handover. Write the next action and responsible person clearly. If an action fails, record the reference and the exact message, then contact the appropriate colleague. Avoid repeatedly clicking a button that creates a booking, invoice or payment.

CHAPTER 02

CRM: enquiries and follow-up

The normal process



In this chapter

Read the CRM dashboard

Record a contact without creating duplicates

Activities, escalation and booking handover

Care Command, channel inboxes and phone

Relationships, web enquiries and advertising

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

2.1 Read the CRM dashboard

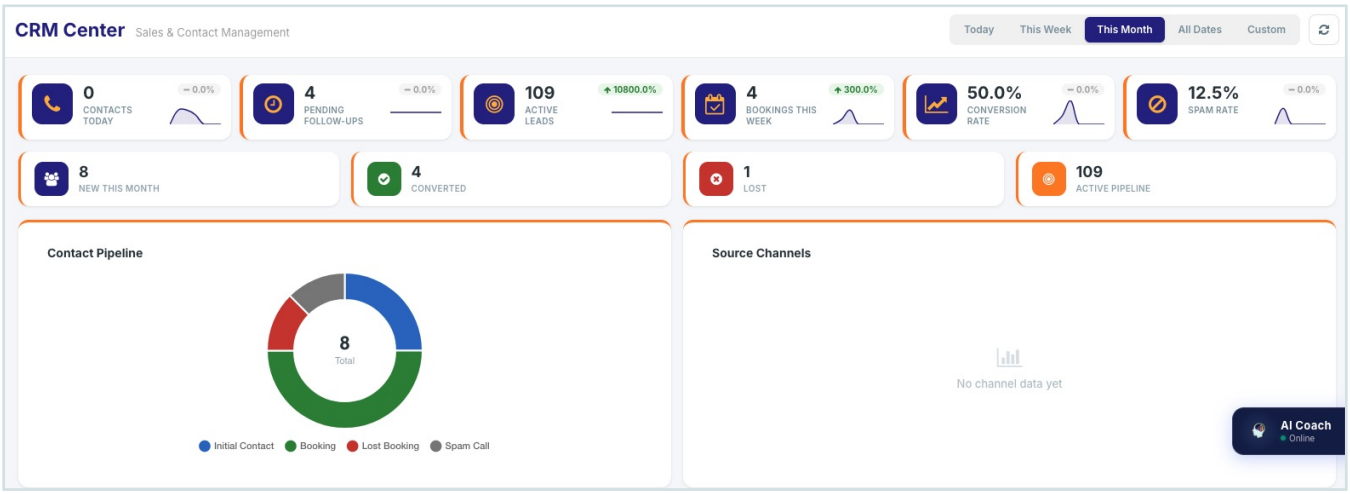


Figure 1. CRM dashboard: period controls, summary cards and pipeline.

Open CRM > Dashboard at the start of a shift. Choose the period and catchment first. The cards summarise contacts, follow-ups, leads, bookings and conversion. The pipeline chart groups contacts by stage; source information shows where enquiries came from. Recent Contacts and Upcoming Bookings provide shortcuts to individual records.

1. Choose Today, This Week, This Month, All Dates or a custom range.
2. Read each card's label. Some cards describe a specific period in their name; do not assume every count uses exactly the same date definition.
3. Open a card or available drill-down to see the underlying records. Check dates and status before drawing conclusions.
4. Refresh after changes and review the follow-up list.

Use the dashboard to decide what needs attention, then do the work on the contact or booking. A high count is a prompt to investigate, not proof that every enquiry needs the same action.

2.2 Record a contact without creating duplicates

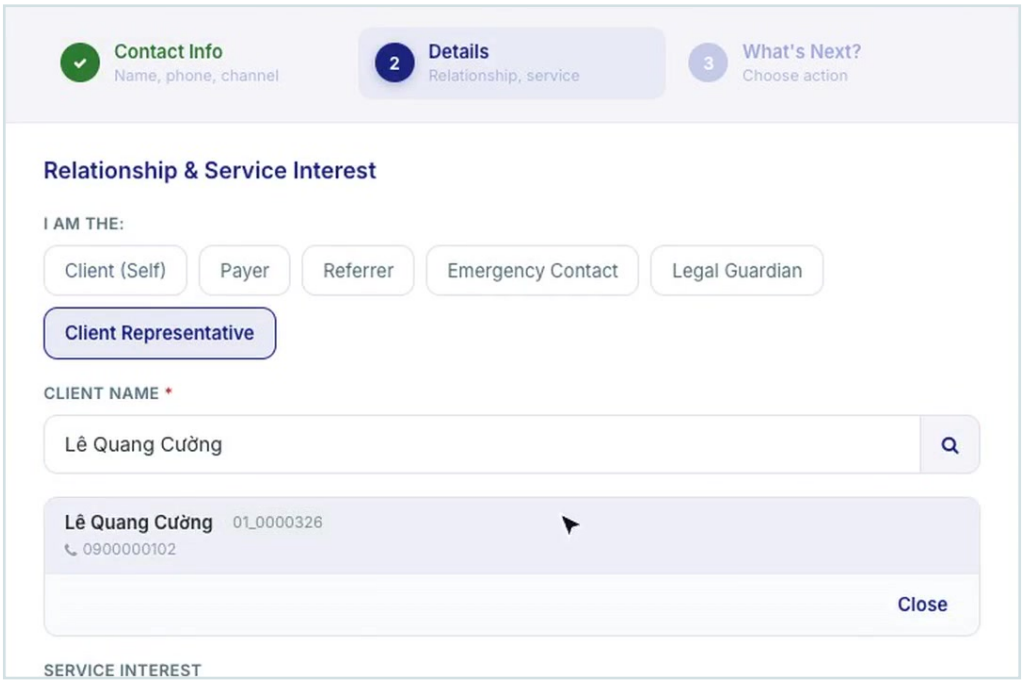


Figure 2. Contact wizard: identify the relationship and select the client.

Open CRM > Contacts > All contacts. Search the phone number and name before creating a contact. A returning caller may already have a client record and earlier bookings.

1. Select New if no suitable contact exists. Enter the caller's details and the enquiry source.
2. Identify who needs the service. If the caller represents someone else, record that relationship and the client's details separately.
3. Record the requested service, location, urgency and notes that help the receiving team. Use approved dropdown values.
4. Save and reopen the contact. Confirm the phone number, catchment and next action.
5. If no appointment is agreed, use Log as Lead or the appropriate follow-up action. The contact can remain in the system without a booking.

Mark genuine junk enquiries as Spam only when appropriate. Do not use Spam to hide a difficult but genuine enquiry. If a booking is lost, use Booking Lost where available and record the reason. The contact's history should explain what happened.

2.3 Activities, escalation and booking handover

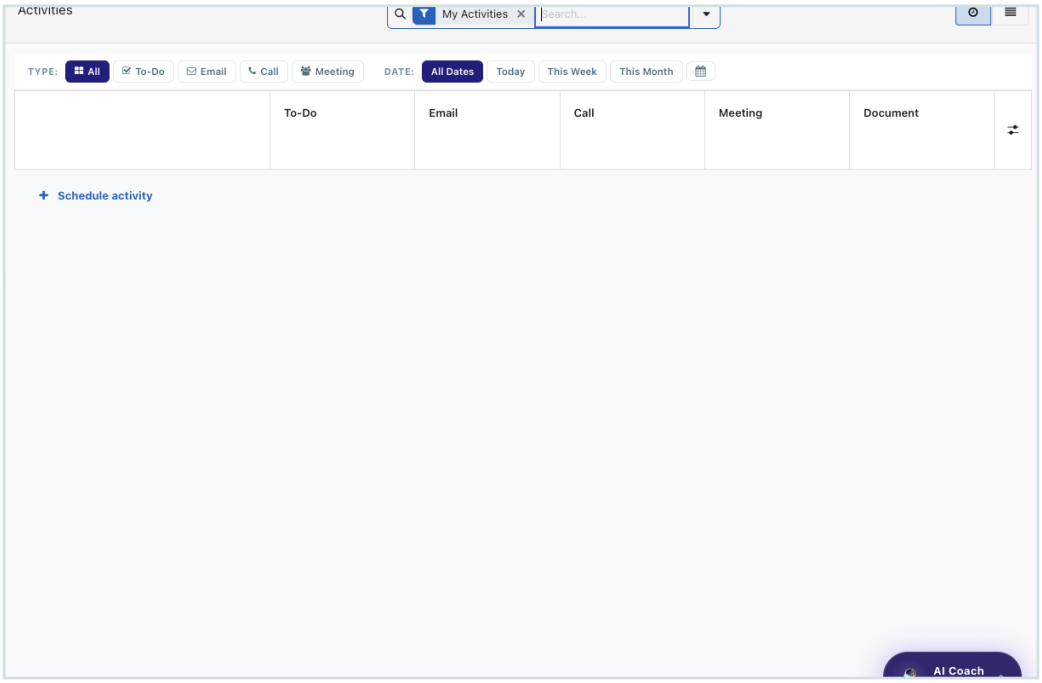


Figure 3. Activities: find the follow-up type, owner and due date.

Action	Use it when
Log Activity / Schedule activity	A call, message, meeting or other follow-up is needed.
Log as Lead	Keep a genuine enquiry for later follow-up.
Escalate	The enquiry needs a manager, senior nurse or doctor.
Book	The client is ready for an appointment; open the booking wizard.
Booking Lost	A booking opportunity has fallen through; record why.

1. Open the contact and choose Log Activity. Set the activity type, person responsible, due date and a clear note.
2. Find the item in CRM > Activities. Filter by type or date, do the follow-up, then record its outcome and complete the activity. Set a new activity if another follow-up is needed.
3. For escalation, choose the appropriate receiving role and explain the question. Check that the enquiry reaches the intended person.
4. For booking, confirm an existing client match before creating a new client. Enter the visit details and review the booking created. The booking reference is the handover to Operations.

If your role cannot create a booking, hand the contact to the authorised booking team. A completed activity closes that follow-up task; it does not automatically mean the whole enquiry has been resolved.

2.4 Care Command, channel inboxes and phone

Care Command brings available conversations into a work queue. Use its filters to find the conversation, open its thread, read the history and identify the correct person before replying or assigning responsibility. A linked contact or booking supplies context; do not assume a similar name proves identity.

Channel Center shows connected channels. Unrouted Contacts contains incoming items that still need a clinic, person or route. Channel messages and Channel audit help authorised staff trace delivery and routing. Availability depends on the clinic's connections. An empty inbox does not prove that every external account is connected.

Phone > Calls shows call activity; Call backs supports return-call work; Recordings opens available recordings; Records from the phone system shows provider history. Check the number and permission before calling or opening a recording. Record the outcome on the relevant contact.

If an AI draft or reply template is available, read it, correct it and deliberately send it yourself. Check the recipient and channel before Send. Do not treat a suggested answer as confirmed clinical advice. Delivery errors should be handed to the channel administrator; avoid sending duplicates while the first message is still pending.

2.5 Relationships, web enquiries and advertising

Relationships records how people are connected, such as a client and family representative. Confirm both people and the relationship before saving. A relationship record does not by itself grant access to health information.

Web & Ads > Web Touchpoints records available website attribution, such as source and campaign. Lead Analysis summarises enquiry flow. Google Ads displays information available from the connected account. These screens help answer which sources bring enquiries; they do not replace the contact's actual service request.

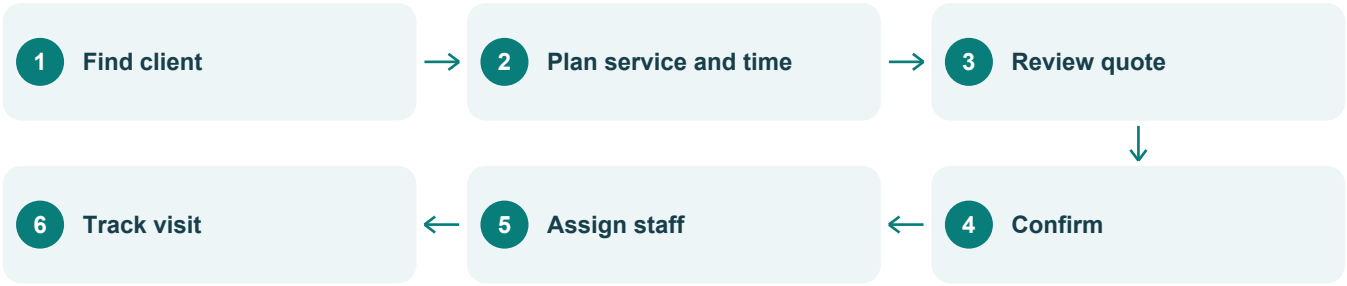
1. Set the relevant dates and catchment where available.
2. Open the linked enquiry to check what happened after arrival.
3. Compare source labels consistently. Missing attribution may mean the source was not supplied.
4. Raise connection or missing-data problems with the administrator. Do not create duplicate contacts to make a chart total change.

Only use a live channel when your clinic has enabled it. Continue to record a walk-in or manually received enquiry through the contact form when appropriate.

CHAPTER 03

Clients, bookings and quotes

The normal process



In this chapter

The client profile and representative

Create one visit or several dates

Read the booking screen and lifecycle

Understand a quote and an added service

Reschedule, cancel and avoid duplicate work

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

3.1 The client profile and representative

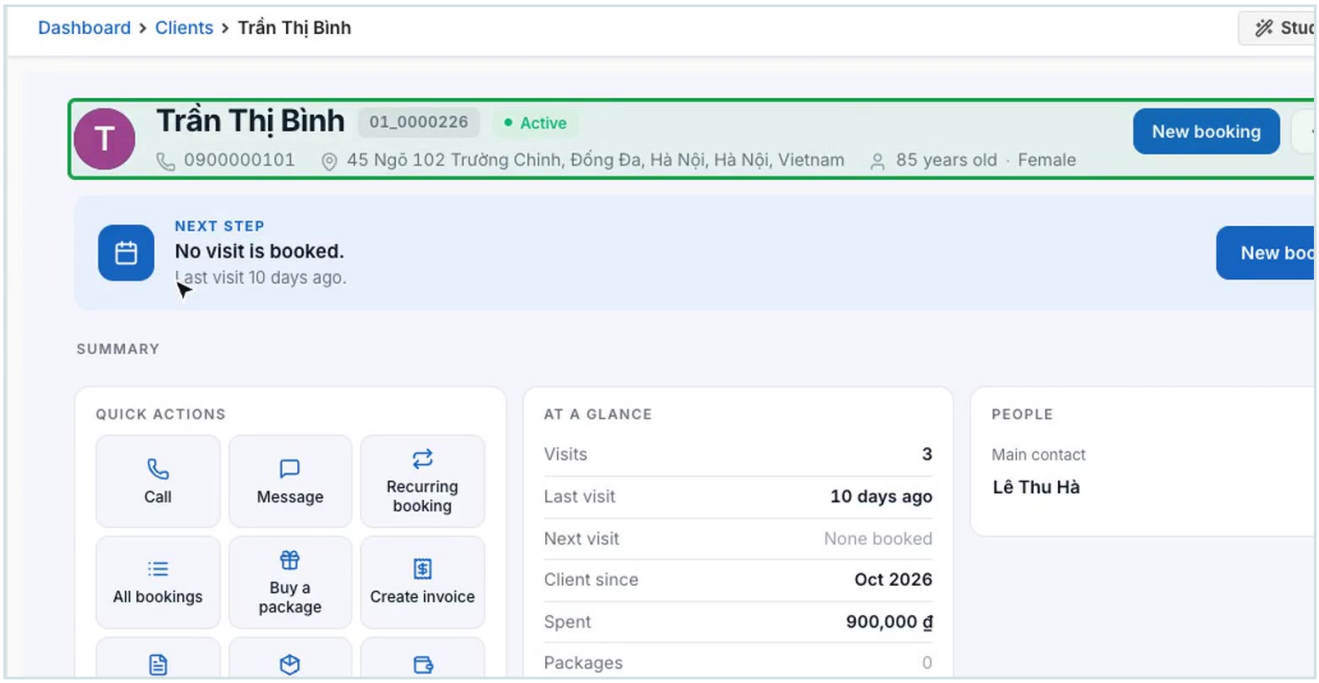


Figure 4. Client profile: review identity and contact information before acting.

Open Operations > Clients and search before creating a record. The profile connects identity, contact details, address, bookings, packages and permitted clinical information. A client may have several visits and more than one representative.

1. Confirm name, phone and another appropriate identifying detail. Do not choose a client solely because the name matches.
2. Check the service address, district, catchment and facility. Review map location and distance if available. A map estimate can be wrong when the address is incomplete; confirm the actual destination.
3. Review the representative and relationship. Keep the person receiving care separate from the person arranging or paying for it.
4. Use the booking or invoice shortcuts to inspect history before creating new work.
5. Update permitted details and save. Reopen the profile to confirm important corrections.

Clinical tabs, care plans, diagnoses, consent and family access appear according to your role and enabled features. Read only the information needed for your work. A profile total is a summary; open the underlying bookings or invoices to investigate a particular amount.

3.2 Create one visit or several dates

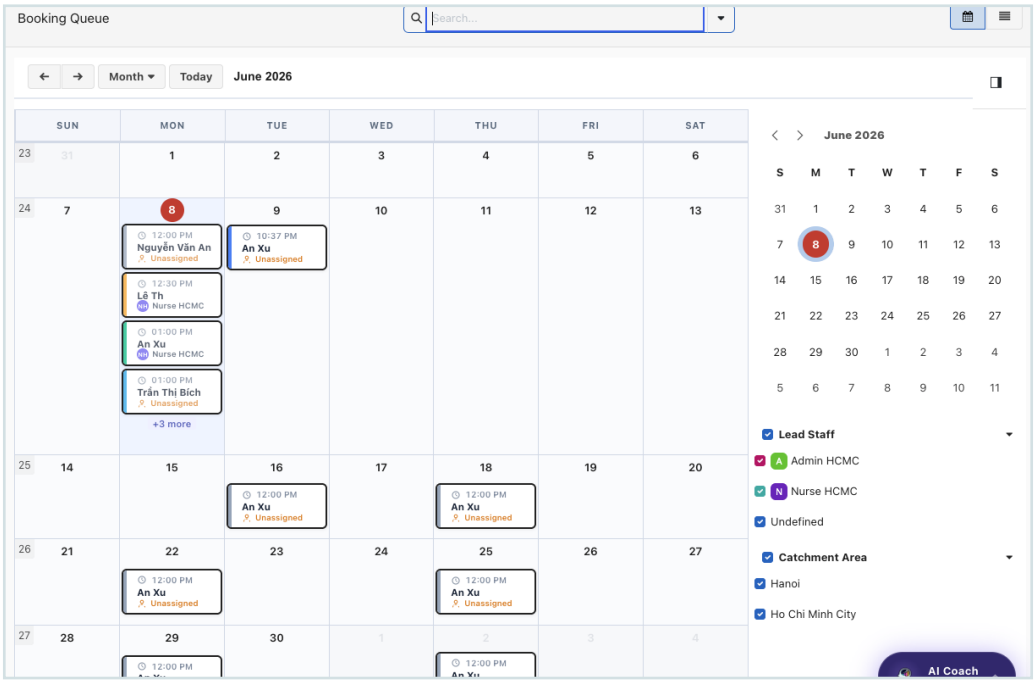


Figure 5. Calendar: choose the period and open a booking to inspect its details.

A booking needs a client, service, place, scheduled time and duration. Create it from the client, contact booking wizard or Operations > Bookings, using the route available to your role.

1. Find the correct client. Confirm whether the service is at home, at the clinic or online.
2. Choose the facility, catchment, service type and exact visit address. Enter the scheduled date, time and expected duration.
3. Add the required service lines, quantities and agreed details. For recurring or multiple-date work, review every date and avoid overlapping or duplicate visits.
4. Save and check the booking reference. Open the quote and review charges before confirming the appointment.
5. Confirm the booking when the details are agreed. Then arrange staff and check acceptance.

Use list view to find records by reference or status; use calendar view to inspect timing. When searching a visit date, use the scheduled-visit field rather than assuming the record creation date is the visit date. After creating several dates, count the resulting bookings and check the first and last date.

3.3 Read the booking screen and lifecycle

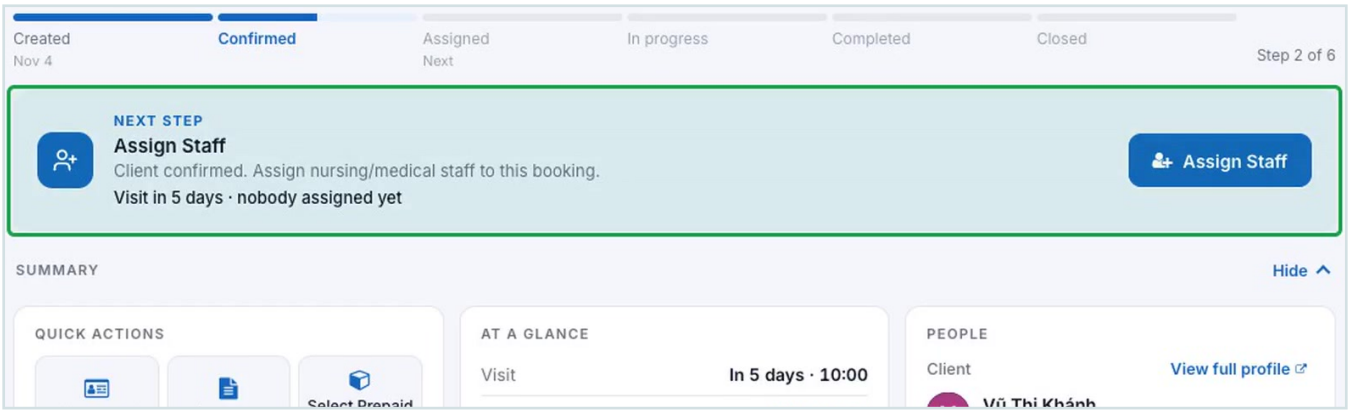


Figure 6. Booking: check status, service details and the next action.

The booking screen brings together client, service, planned time, staff, clinical work and financial links. Its status tells you which stage has been reached. The Next Best Action panel offers an action appropriate to the current state; check the details before using it.

Stage or control	Meaning
Draft / Created	Details are being prepared.
Confirm Booking	Confirm the agreed appointment details.
Assigned	Staff have been attached; check whether the assignment is accepted.
Start Service / In Progress	Actual service has started and timing is recorded.
Complete Service / Completed	Service work is finished; check notes, closing answers and invoice separately.
Completed-Pending-Invoice	Service is finished but invoice work remains.
View Quote / View Payments / View Assignments	Open the related records.
Close	Finalise the booking when the required work has been completed.

The system may create or link an invoice during completion, depending on the workflow. Open the linked invoice before using Create Invoice so you do not create a duplicate. A support staff member finishing their own assignment is not always the same as the lead completing the overall visit.

3.4 Understand a quote and an added service

A quote explains the planned charge. The amount can depend on service, quantity, staff or service category, catchment, date, time, duration, travel and approved discounts. An invoice is the bill produced from the final charge.

1. Open View Quote from the booking. Check the client, currency and all service lines.
2. Compare quantities, unit prices, extra time, travel charges and discounts with the agreed service. Check the breakdown rather than only the grand total.
3. If a service is added during the visit, use Add Service or the available service-line action. Select the actual service and quantity; do not disguise it as a discount or a manual payment.
4. Save or recalculate using the available quote action. Review the updated amount and any resulting invoice lines.
5. Tell the responsible office colleague if an invoice already exists and an adjustment is required.

Do not assume that a fee in an example is today's clinic price. A missing tariff, incorrect catchment or wrong visit time should be investigated before billing. Ask the authorised pricing administrator to correct configuration; regular users should not change the master price list to make a single quote look right.

3.5 Reschedule, cancel and avoid duplicate work

To reschedule, open the existing booking and choose Reschedule. Check the new time, duration and available staff. If changing the nurse, confirm the old and new assignments. Reopen the booking and inspect the calendar after saving. Where schedule dragging is enabled, moving a block is a real reschedule, not just a visual change; review the confirmation and warnings.

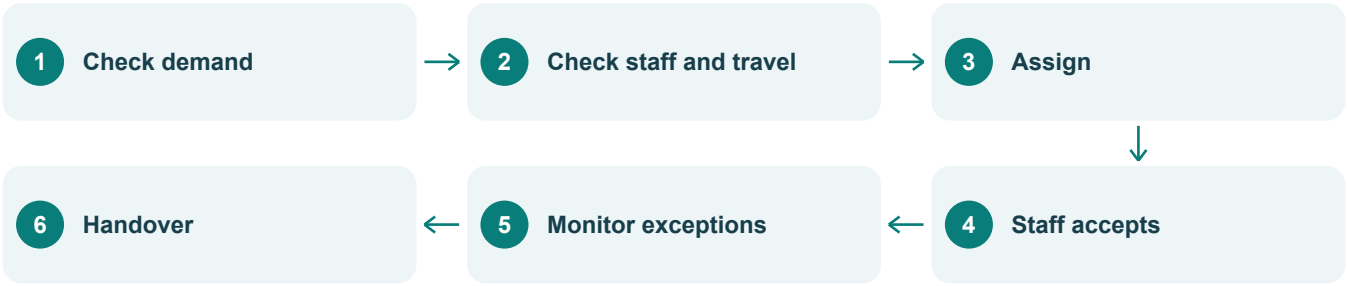
To cancel, use Cancel Booking and record the real reason. Check that assignment and notification outcomes agree with the cancellation. Do not delete the client because one visit is cancelled. For work already completed or billed, ask Operations and Finance to handle the appropriate correction instead of trying to hide the history.

If the browser reports an error or times out, search the booking reference before creating it again. If the appointment moved successfully but a message failed, fix the communication separately. The booking, assignment, invoice and outgoing message each have their own record of what happened.

CHAPTER 04

Operations: staffing and the working day

The normal process



In this chapter

Use the Operations dashboard

Staff Schedule, Time Off, Workload and Routes

Exceptions, cash handover and staff development

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

4.1 Use the Operations dashboard

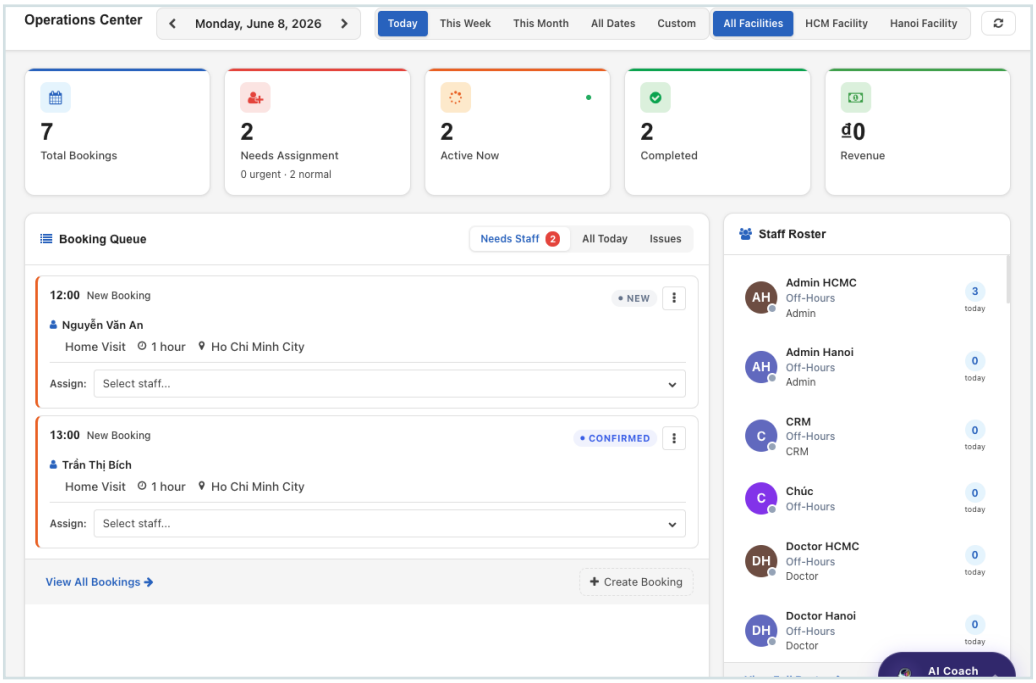


Figure 7. Operations overview: review the queue, staff and timing together.

Start in Operations > Dashboard. Choose the date, facility and catchment. Review bookings needing staff, visits in progress and completed work. The queue and timeline help you see the next action without opening every booking.

1. Open an unassigned booking and confirm the required service, time and location.
2. Check the staff roster, availability and workload. A free-looking slot is not enough if the employee lacks the required skills or travel time.
3. Use Assign Staff or the inline assignment control. Check who is lead and who is support where multiple staff are needed.
4. Verify acceptance. An offer or assignment is not proof the staff member has seen and accepted it.
5. Review issues throughout the day and arrange a replacement when someone declines.

Use the dashboard's links to open the actual booking. Refresh to see current information. A visit can be clinically finished while finance still has work to do; follow the correct queue for each outstanding item.

4.2 Staff Schedule, Time Off, Workload and Routes

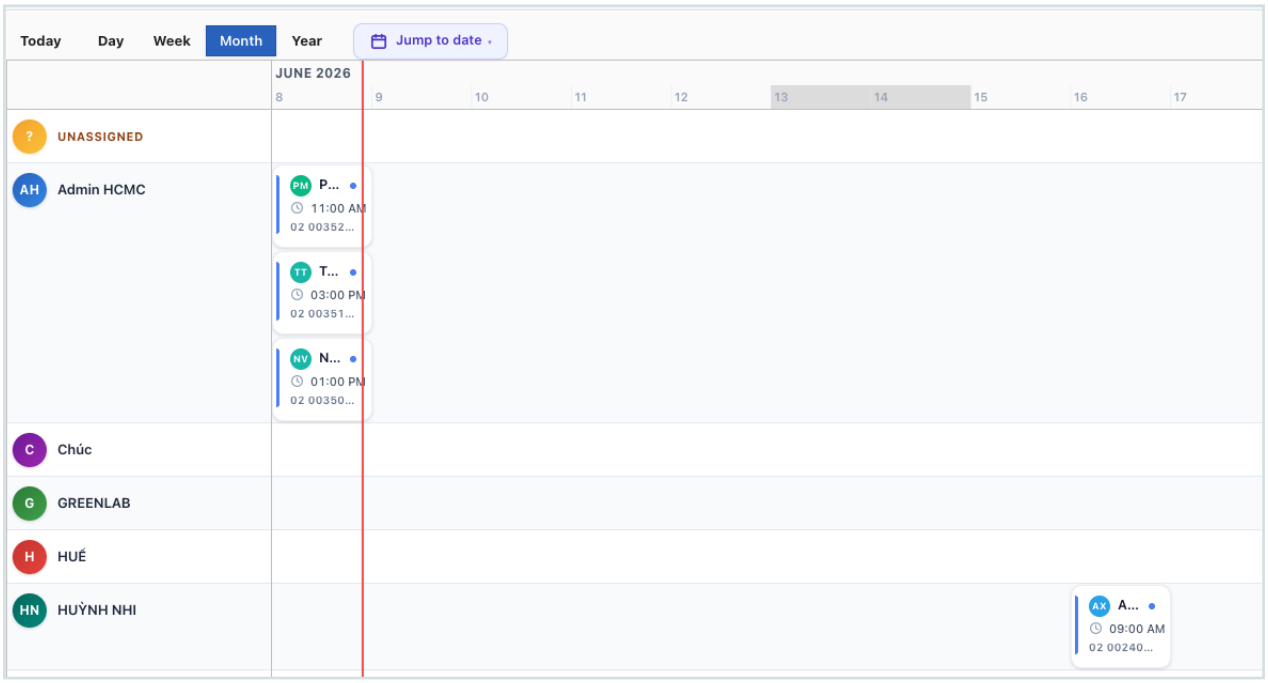


Figure 8. Staff Schedule: check the visit time against the available staff.

Schedule > Staff Schedule shows work against staff and time. Use its date and view controls to find the right day. Open a block to check the linked booking. If draw-to-create is enabled, selecting a free time range starts booking creation; review the form before saving.

Time Off records absence. Workload summarises how much work staff have. Routes checks whether travel between visits is feasible. Route warnings help planning; they do not automatically move visits or guarantee arrival times.

1. Compare the requested visit with working hours, leave and existing assignments.
2. Leave realistic travel time between locations. Investigate a route warning rather than ignoring it because there is no time overlap.
3. Assign an eligible staff member and check the final schedule.
4. After a change, verify all staff on a multi-person visit are still aligned with the visit time.

If a visit is declined, open the assignment or booking and choose a replacement. Confirm the replacement has accepted and the old assignment no longer appears as active work. Notify the responsible team of the final arrangement.

4.3 Exceptions, cash handover and staff development

Exceptions > First-visit offers helps follow assignment offers. Timecard mismatches highlights differences between visit times and attendance. Use Visit Times and Keep Attendance represent different decisions; check the evidence and your authority before choosing either. Neither button should be used simply to clear a warning.

Collections tracks field cash awaiting handover. Count the money with the staff member, match the client and payment references, then use the authorised receiving action. A handover confirms movement of money already collected; it is not a second client payment.

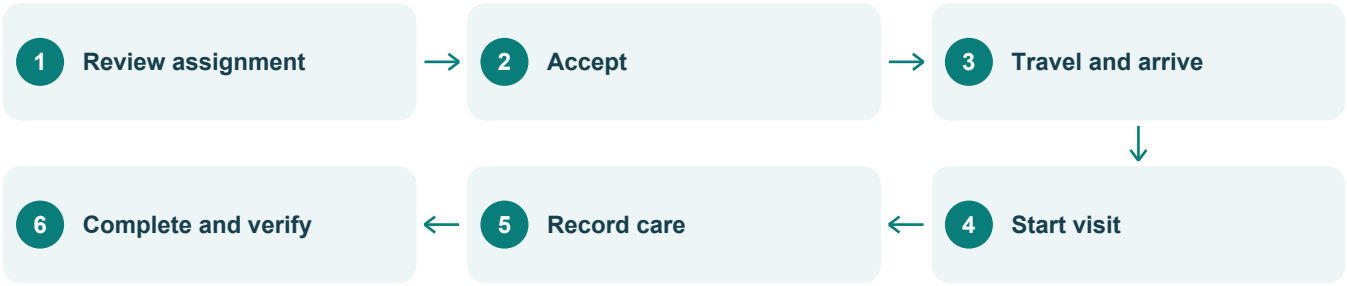
Development > Employee development and Coaching support staff plans, objectives and coaching sessions. Open the person's plan, review agreed objectives and due dates, and record progress. Start Session and Complete Session track the coaching meeting. AI summaries or suggested actions need human review before use. Keep staff development information within its permitted audience.

At the end of the day, check unresolved offers, unfinished visits, timing exceptions, pending cash and important follow-ups. Record the next responsible person so work continues safely across shifts.

CHAPTER 05

The nurse and doctor mobile app

The normal process



In this chapter

Open the app and find your visits

Carry out a visit step by step

Notes, voice input, signing and verification

Patients, calls, family messages and profile

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

5.1 Open the app and find your visits

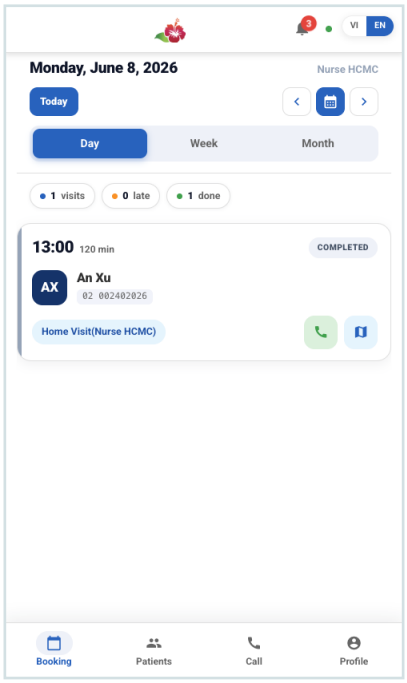


Figure 9. Mobile day view: select the date and open the assigned visit.

Open https://carejiox.com/health_pwa on your phone, or open the installed app. Sign in with your own staff account. If the browser offers installation, use its Install or Add to Home Screen option. Installation creates a convenient shortcut; it does not create a new account.

Booking is the main work tab. Use Today, previous/next arrows or the date picker, then choose Day, Week or Month. Open a visit card to see its details. Confirm client, date, address, service and your assignment before acting.

The bell shows available notifications and assignment offers. Accept confirms you can take the work; use the decline action and reason if you cannot. Check with Operations if the expected visit is missing. An empty day does not mean the clinic has no appointments.

Use EN / VI to choose the available interface language. Display modes can improve visibility in sunlight or increase touch targets for glove use. Help opens the relevant written guide and video beside your work; online content needs a connection.

5.2 Carry out a visit step by step

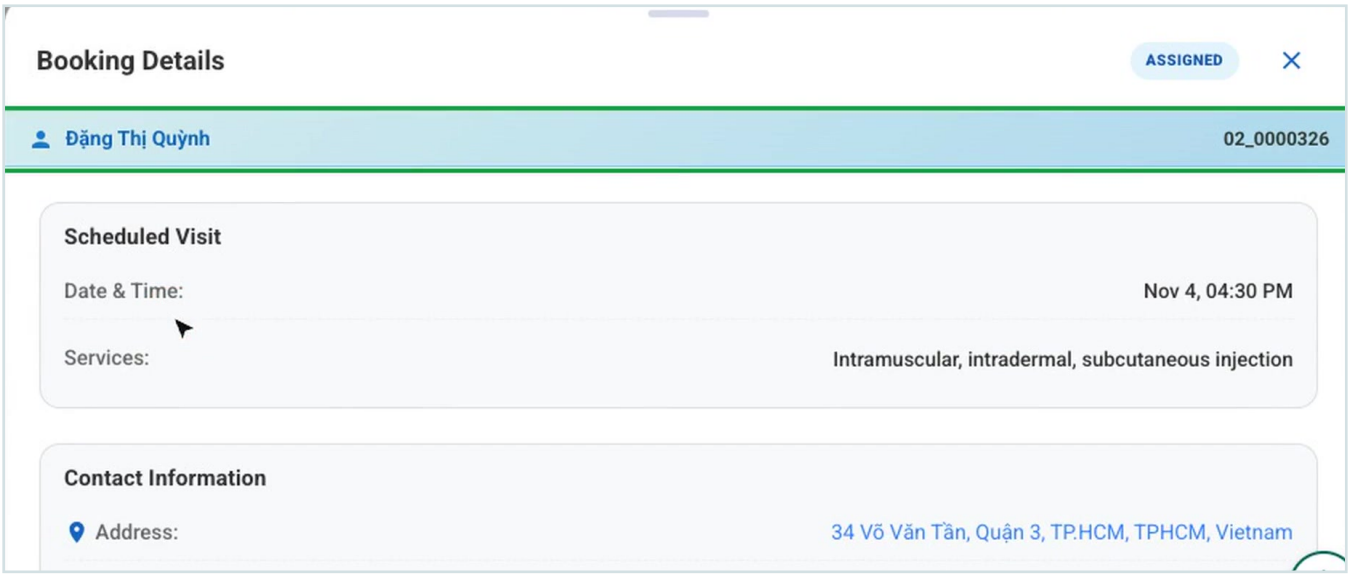


Figure 10. Visit detail: confirm client, time, service and destination.

1. Open the assigned visit. Read the client details and service instructions. Use Call or Map only after checking the intended person and destination.
2. Use On my way where available when you actually begin travelling. It records a travel event; it does not start the service timer.
3. At the correct location and when ready to provide care, choose Start Service. Supply required location permission or verification information if prompted. Confirm the visit is In Progress.
4. Open Clinical Notes and record the work in the correct visit. Save, close the note panel and reopen it to confirm the note is retained.
5. Complete any required observations, assessments, care-plan tasks or medication records that apply to this visit. Add an authorised extra service through the service action when needed.
6. Answer the closing questions accurately. Choose Complete / Complete Service only when your work and required records are finished.
7. Reopen the visit to check completion and the linked invoice, if your role can see it. If the status remains pending, read the message and ask Operations for the remaining action.

If multiple people attend, follow the lead/support responsibility shown in the assignment. Do not start or finish another person's work to remove an error.

5.3 Notes, voice input, signing and verification

Write factual observations and care performed in the correct client's visit. Voice input, when enabled, turns your recorded observations into draft text. Read the entire transcript, correct names, numbers and meaning, and save the reviewed note. A recording or transcript is not a signed clinical record.

Finalize & Sign is a separate action for an authorised clinician. Check the content before signing. A final note is protected from ordinary editing; use the permitted addendum process for later corrections rather than overwriting the original.

Visit verification may record check-in/out times, location and signatures. If location is unavailable or appears wrong, explain the issue and contact Operations. Do not invent coordinates or another person's signature. Online visits follow the configured telehealth workflow.

For a connection problem, read the app's connection or pending status. Some data can be available locally, but videos, live calls and server actions need connectivity. Reconnect, use Sync Data where available, and verify the record on reopening. Do not log out, clear browser data or reinstall while unsent work remains. If unsure whether an action was saved, check before entering it again.

5.4 Patients, calls, family messages and profile

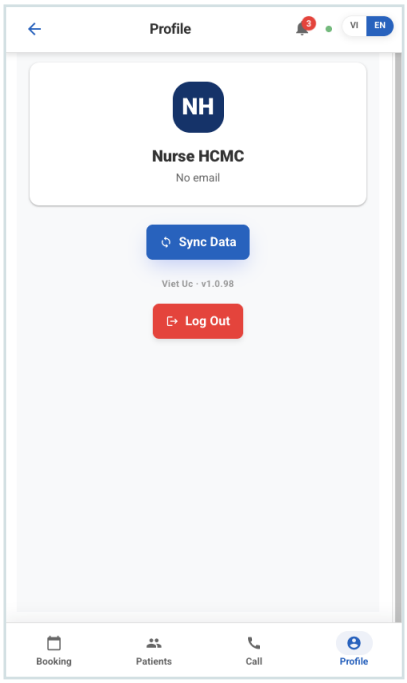


Figure 11. Profile: verify the signed-in person before sync or sign-out.

Patients provides a searchable list within your access. Open the correct person and review permitted information. Call opens the clinic contact route; the Call button on a visit refers to that visit's contact. Check the number before dialling.

Family Messages, where enabled on the visit, lets the care team read and respond to permitted family communication. Review the recipient and the message before sending. A post-visit update may be offered after completion; check its content and consent requirements. Do not promise that a message was delivered merely because it was drafted.

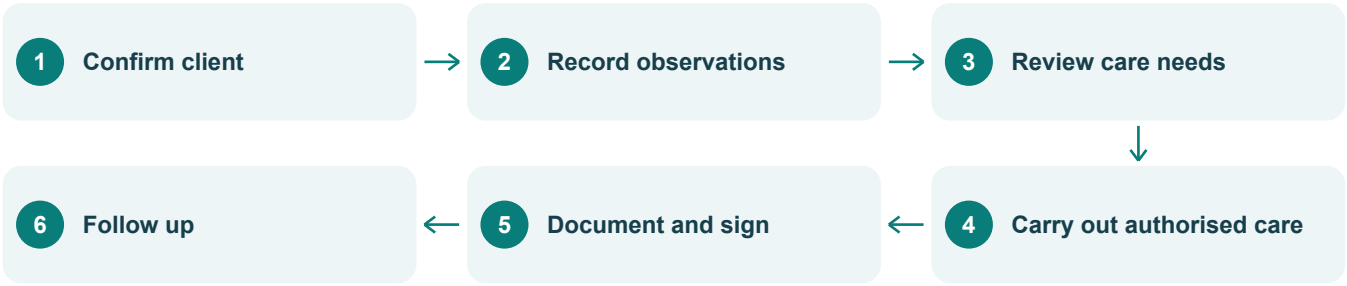
Profile shows your signed-in identity and available sync/logout controls. Use Sync Data to request a refresh and inspect pending work. Log Out ends the session and can clear local patient information, so first confirm your changes have reached the server.

Notifications depend on device permissions and the clinic's delivery setup. If notifications are blocked, use the app's instructions and continue to check your work list manually. A missing notification does not cancel an assignment.

CHAPTER 06

Clinical records and care coordination

The normal process



In this chapter

Vitals and observations

Care Intelligence: worklist, alerts and NEWS2

Care plans and visit tasks

Medication orders and the administration register

Assessments and clinical forms

Notes, diagnoses and coding review

Consent and permission to share

Report an incident and follow corrective actions

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

6.1 Vitals and observations

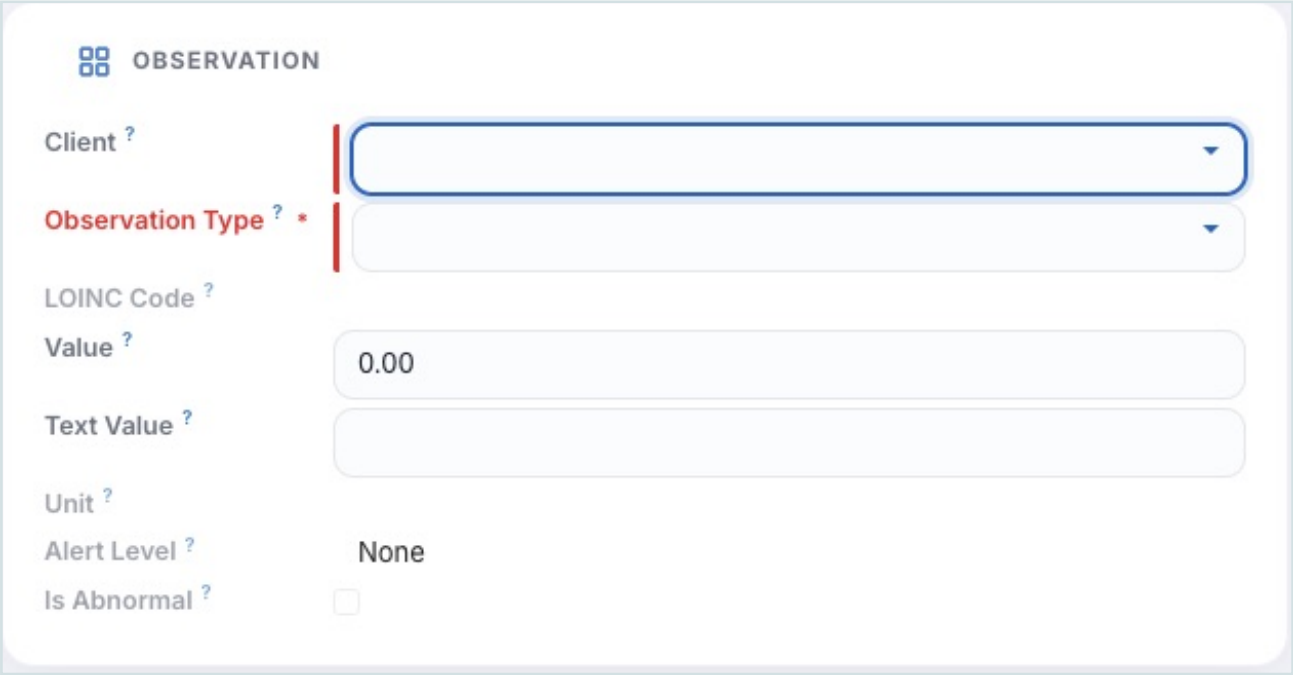


Figure 12. Observation entry: select the client and measurement type, then enter the actual value.

Clinical > Observations stores structured measurements. Unlike a free-text note, each observation has a type, value, unit and time. This makes it possible to review trends and use the measurement in monitoring.

1. Open the correct client or visit and create an observation through the available action.
2. Choose the observation type. Enter the measured value in the displayed unit; do not guess or copy a previous measurement as a new reading.
3. Check the effective date and time, performer, visit link and relevant method or position.
4. Save and inspect the result, including any abnormal or alert indicator. If information was entered incorrectly, use the permitted correction or Entered in Error process rather than concealing the error.
5. Follow the clinic's escalation procedure for concerning findings. Recording a value is not the same as notifying the responsible clinician.

The software supports recording and review. It does not decide the care to provide. Missing or old measurements should not be treated as current normal results.

6.2 Care Intelligence: worklist, alerts and NEWS2

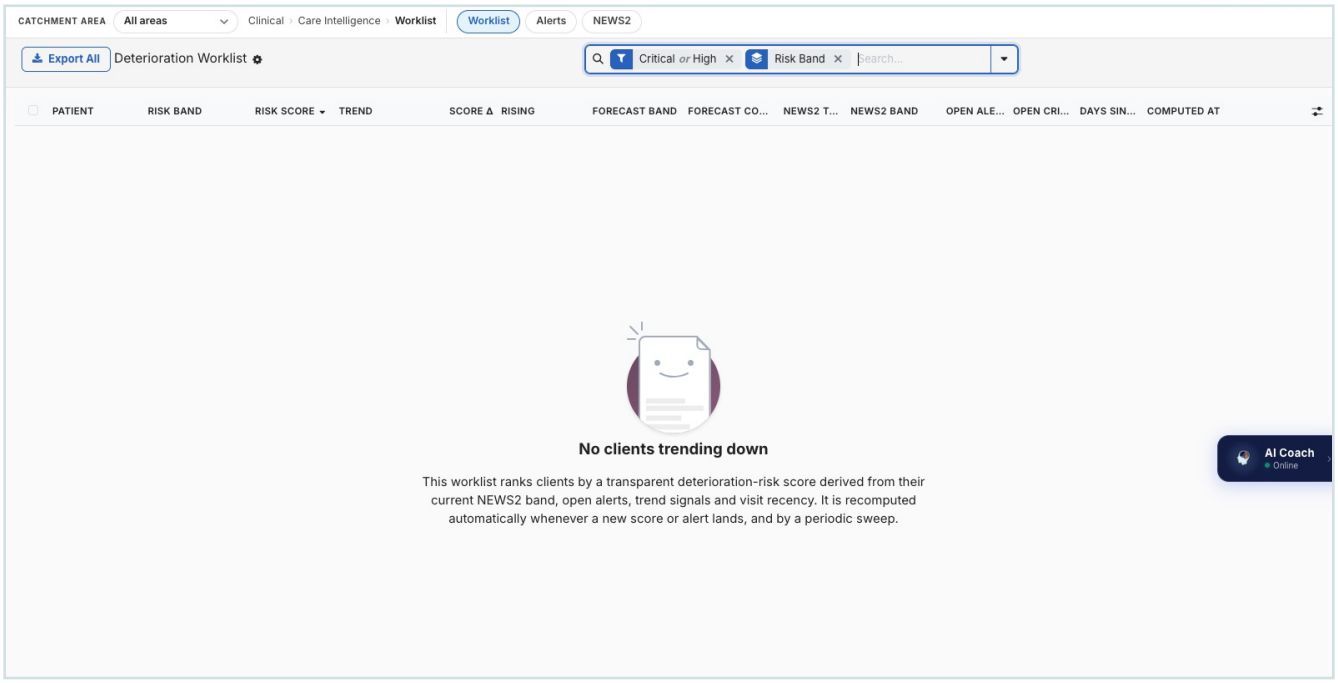


Figure 13. Care Intelligence: use the worklist, alerts and NEWS2 tabs for review.

The Worklist brings together clients who may need review, based on available monitoring signals. Open a row to inspect the reasons, dates and supporting observations. A ranking helps organise review; it is not a diagnosis.

Alerts has separate states. Acknowledge means somebody has taken responsibility for reviewing it. Resolve records that the issue has been addressed. Dismiss records that the alert is not applicable, with the required explanation. Acknowledging does not mean the client's condition has improved.

NEWS2 shows the available score and its component measurements. Review when those measurements were taken and whether the required components are present. Do not interpret missing data as a reassuring score. This manual explains the screen, not clinical thresholds or treatment decisions.

1. Review the highest-priority or newest items according to your clinic's procedure.
2. Confirm the client and evidence, then contact the appropriate clinician when needed.
3. Record the action taken and update the alert through the permitted button.
4. Arrange and document follow-up. Device or data-source problems should be reported separately from the clinical issue.

6.3 Care plans and visit tasks

A care plan describes goals and planned activities over time. A visit task is a particular item to be completed during a visit. Open the client's Care Plans tab or the care-plan action available to your role. The visit's Visit Tasks tab shows the related work.

An authorised clinician prepares the plan, goals, activities and review dates. Activate makes the plan active. Start Review and Confirm Review record a planned review. Complete finishes the plan; Cancel Plan stops it through the appropriate process.

During a visit, read each task and record the actual outcome. Mark Done means the task was performed. Mark Not Done records that it was not performed; provide the reason requested. Do not mark all tasks done simply to finish the screen. Reset to Pending is for a permitted correction, not for erasing what happened.

Check the client, active plan and visit before recording a result. If the plan is outdated or a task is unsuitable, contact the responsible clinician and document the issue. Changing a care plan is different from recording that a visit task was completed.

6.4 Medication orders and the administration register

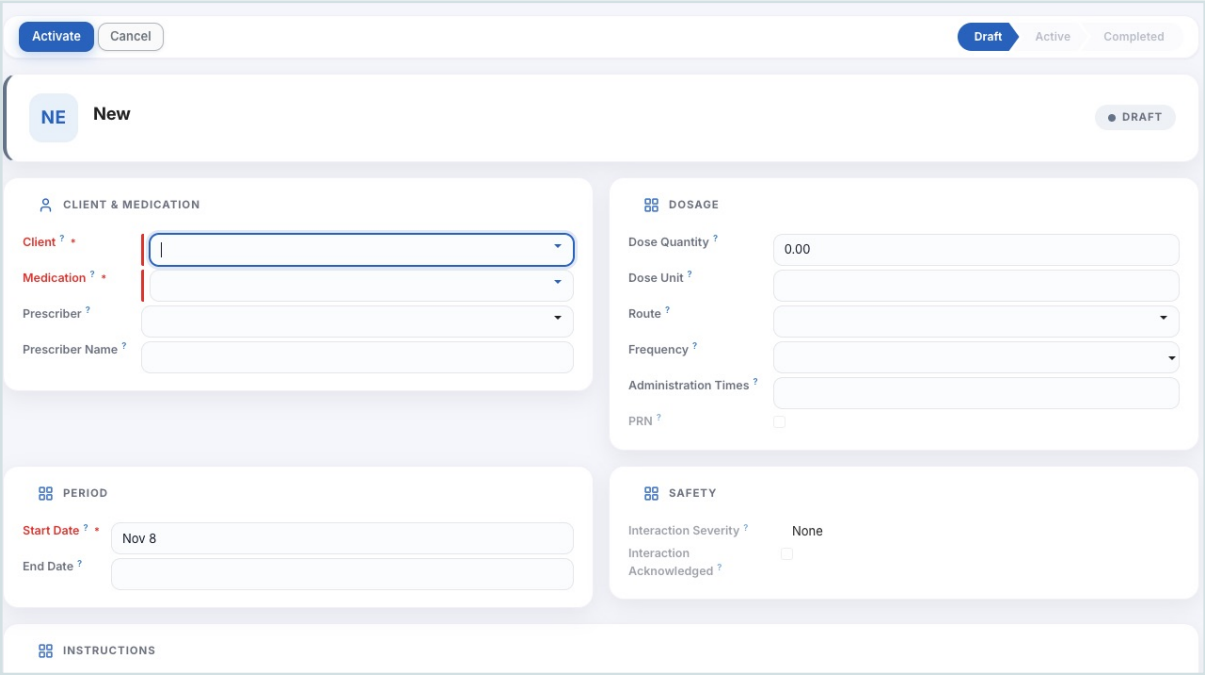


Figure 14. Medication order: review status and instructions before activation.

Clinical > Medications > Orders contains medication instructions. The Administration Register records individual planned or actual administrations. An order is not evidence that a dose was given.

Authorised prescribers review the client, medication, dose, route, frequency, start/end dates and instructions. Activate brings an approved order into use. Put On Hold pauses it; Resume restarts it; Complete or Cancel ends the order as appropriate. Interaction warnings and acknowledgement require clinical review, not a routine click.

For a scheduled administration, confirm the correct client, visit, medicine and order before recording what happened. Record Given documents an administration actually performed. Record Not Given or Record Refused documents a different outcome and its reason. Cancel Slot is a scheduling action, not proof of administration.

Read the saved outcome and time in the register. If you made an error or the order conflicts with the care instructions, stop and refer to the responsible clinician through the clinic's process. This screen records authorised medication work; it is not a guide to choosing medicines or doses.

6.5 Assessments and clinical forms

Clinical > Assessments and the visit's Assessments tab hold completed or in-progress forms. A published template defines the questions, answer types and any scoring.

1. Open the correct visit and choose New Assessment where available. Select the applicable template and confirm the client.
2. Answer every relevant question using the requested type: text, number, date, selection, photo or signature. Use the units shown.
3. Review required questions and any score or band displayed. A calculated score is based on what was entered; incorrect inputs produce misleading results.
4. Save unfinished work as permitted. Choose Complete only after reviewing the answers.
5. Reopen the assessment and verify the status and links to any extracted observations.

Do not change the template to avoid a question. Ask the responsible clinician or form administrator if a question is inappropriate or a required option is missing. A signature must be supplied by the appropriate person, not copied from another record.

6.6 Notes, diagnoses and coding review

Clinical > Notes groups Unsigned Notes, Voice Notes and Coding Review. Unsigned Notes helps authorised clinicians find notes needing final review. Voice Notes shows available transcription jobs and their status. Check a failed or pending job before recording again.

Coding Review holds suggested medical codes. An authorised reviewer reads the source note and proposed code, then uses Approve or Reject. Approval is a deliberate human decision; an AI suggestion is not automatically a confirmed diagnosis.

The client's Diagnoses tab links conditions to supporting information. Mark Resolved, Mark Inactive and Reactivate change the condition's clinical status. Only the appropriate clinician should make those changes. Read the evidence and date before interpreting the problem list.

For a signed note that needs correction, use the authorised addendum route. Keep the original account of events intact and explain the correction. If you cannot see a signing or diagnosis action, ask the responsible clinician rather than asking for broad access just to complete the screen.

6.7 Consent and permission to share

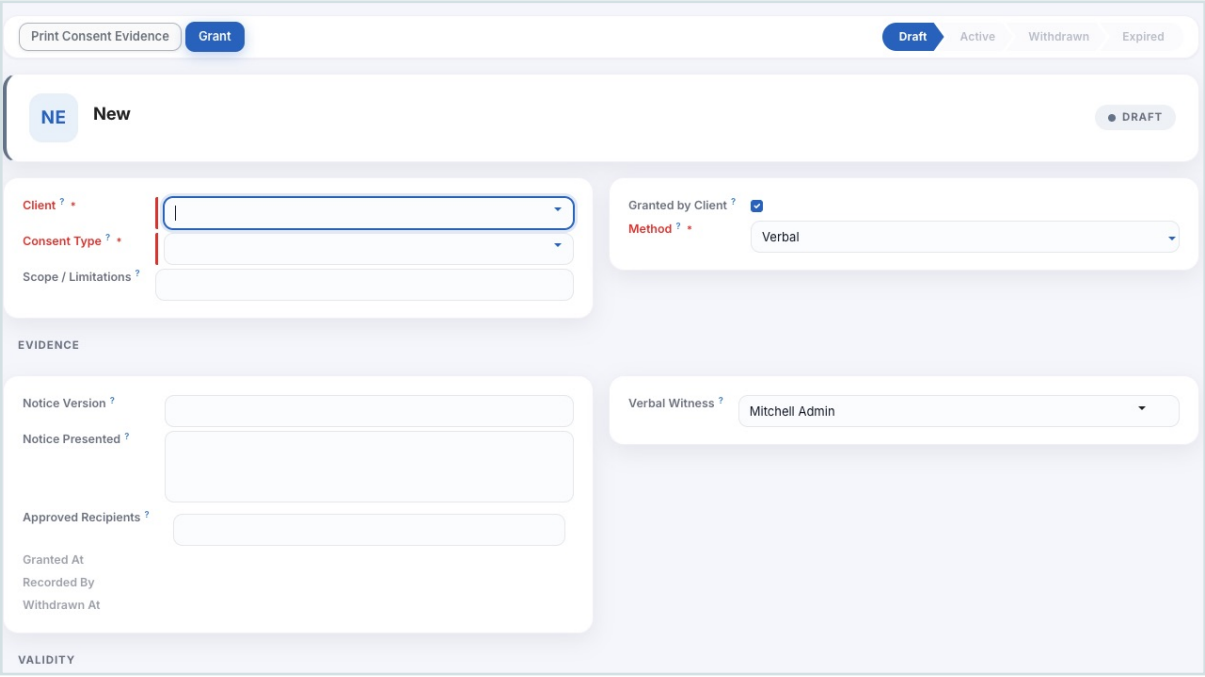


Figure 15. Consent form: identify purpose, person and supporting evidence.

Clinical > Consents helps review consent records, including those near expiry. Consent describes permission for a particular purpose; it is not a permanent blanket permission for every activity.

1. Confirm the client, purpose, person giving consent, relationship and method.
2. Record the required evidence, dates and validity details. Read the information with the appropriate person through the clinic's process.
3. Use Grant only when valid consent has actually been obtained and you are authorised to record it.
4. Check the active state and expiry before the related action. Withdraw records withdrawal; it does not erase the history.
5. Print Consent Evidence can produce a record of the captured evidence where permitted.

A relative listed on the profile does not automatically have permission to receive all records. If a family message, portal link or other action is blocked for consent, resolve the underlying permission with the responsible team. Do not bypass it by sending through an unrelated channel.

6.8 Report an incident and follow corrective actions

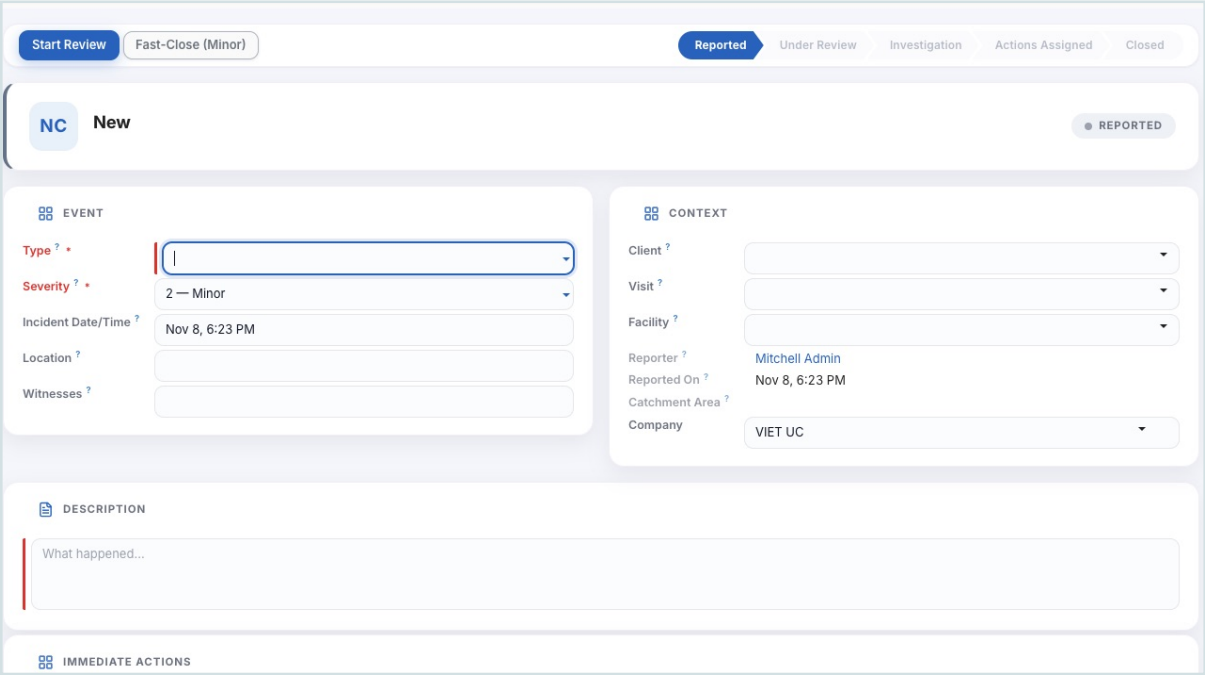


Figure 16. Incident form: record the event and use the appropriate review stage.

Use Clinical > Incidents > Register to record an event affecting care or service safety. Attend to immediate needs through the clinic's normal response process first; entering a form is not an emergency notification.

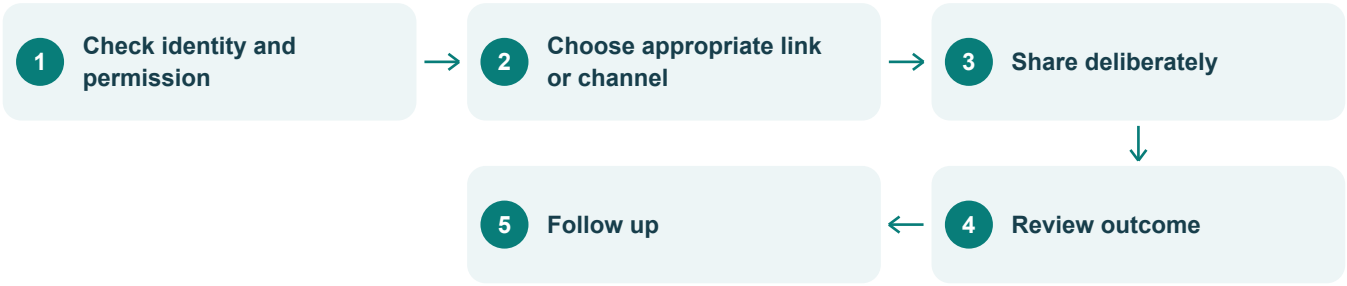
1. Create the incident with the correct client, visit, date/time, category, severity and factual description. Record immediate action and relevant evidence.
2. Save and notify the responsible reviewer through the agreed route.
3. The reviewer uses Start Review, then Start Investigation where needed. Record findings and assign corrective actions with an owner and due date.
4. Use Corrective Actions to track work. Start and Mark Done describe the action's progress; attach or record evidence of completion.
5. Close the incident only when the appropriate review and actions are complete. Fast-Close (Minor) is a restricted shortcut for qualifying minor cases, not a way to skip review of a serious event. Reopen is used when further work is needed.

Analysis summarises incident information. Check the date range and classification before comparing totals.

CHAPTER 07

Remote visits and family communication

The normal process



In this chapter

Telehealth and self-booking

Family Inbox, visit links and My Care

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

7.1 Telehealth and self-booking

For an online visit, confirm that the booking has the correct service type and time. Open the session or use Join Video when available. Check microphone/camera permissions and the intended participant before discussing care. The patient may enter through the session's waiting-room link. If the video provider is not configured or the connection fails, contact the clinic and record the agreed alternative; do not mark a visit complete merely because the call screen was opened.

Self-booking uses an invitation for a specific client. When the Send self-booking link action is available, confirm the recipient and the allowed services or dates before sending. The client uses the link to choose an offered slot. Check the resulting booking in Operations and arrange staff as required.

A link is not evidence that it has been used. An offered slot is not the same as a completed visit. Use the booking and session status to confirm progress. Do not copy one client's private link to another person.

7.2 Family Inbox, visit links and My Care

Family Inbox supports permitted two-way communication between the care team and a client's family. Open the thread, confirm the linked client and recipient, read earlier messages, then reply. Mark read records that a message was seen; it does not mean the requested work is complete.

A family visit link provides a limited view of an individual visit. My Care is a client portal whose available information depends on the enabled features and permissions. Treat both as private links. Share only through the approved route with the intended recipient. If a link is wrong, expired or no longer appropriate, ask the authorised colleague to revoke or replace it.

1. Check identity, relationship and active consent before sharing.

2. Review the content of a visit update or outgoing reply.

3. Send once and inspect the delivery or thread status.

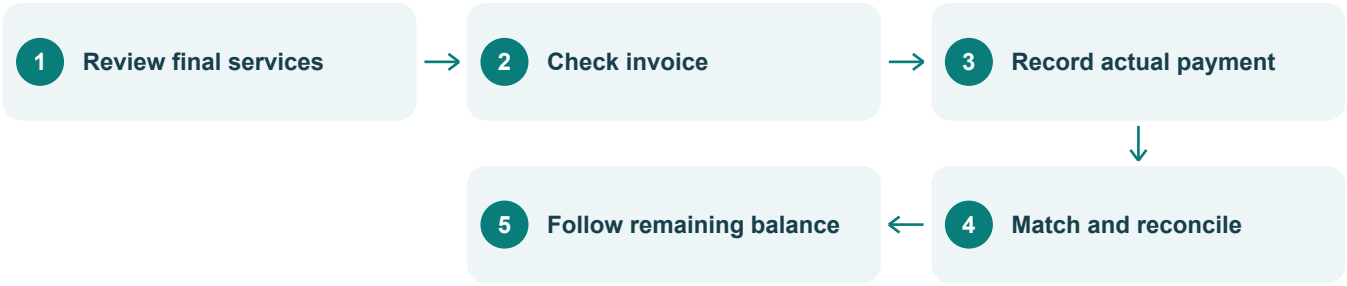
4. Record any action needed inside the client's care or booking record, not only in chat.

A family message is not automatically an instruction to change care. Refer clinical questions and urgent concerns to the responsible clinician through the clinic's established process.

CHAPTER 08

Finance: invoices, payments and balances

The normal process



In this chapter

Read Finance and invoice statuses

Review and issue the correct invoice

Receive a payment or part payment

Field cash, office receipt and reconciliation

Receivables, refunds, credit and packages

VAT log, e-invoices and BHYT claims

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

8.1 Read Finance and invoice statuses

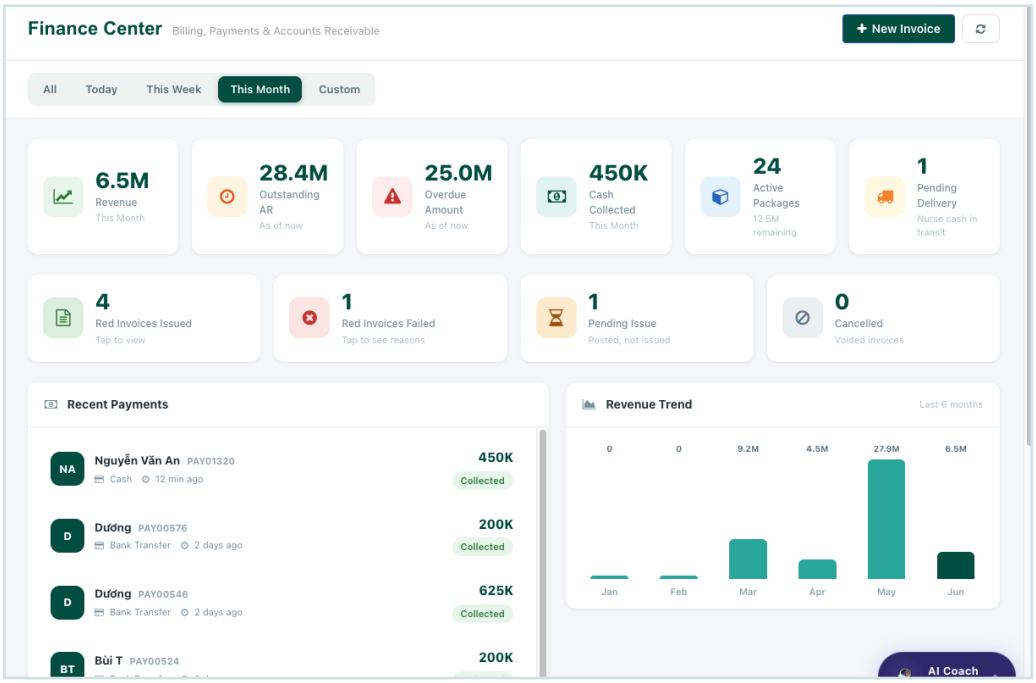


Figure 17. Finance overview: inspect the period and separate money owed from cash received.

Finance > Dashboard summarises invoices, receivables, payments and field cash. Set the period first. Use an available card or list link to inspect the underlying records. Finance > Invoices is the working list of bills.

Label	Meaning for your work
Draft	The bill is being prepared and must be reviewed.
Posted	The invoice has been confirmed in the accounting record.
Partial	A payment has been applied but a balance remains.
Paid	The invoice's recorded balance has been settled.
Overdue	A balance remains after the due date.
Cancelled	The document has been cancelled through its permitted workflow.

Document state and payment status are different. An invoice may be posted and only partly paid. Open it to check the client, invoice reference, due date, currency, service lines, taxes, total and amount due. An e-invoice's provider status is another separate status.

Do not infer accounting or tax treatment from a dashboard label alone. Use the clinic's approved finance process and the detail shown on the underlying document.

8.2 Review and issue the correct invoice

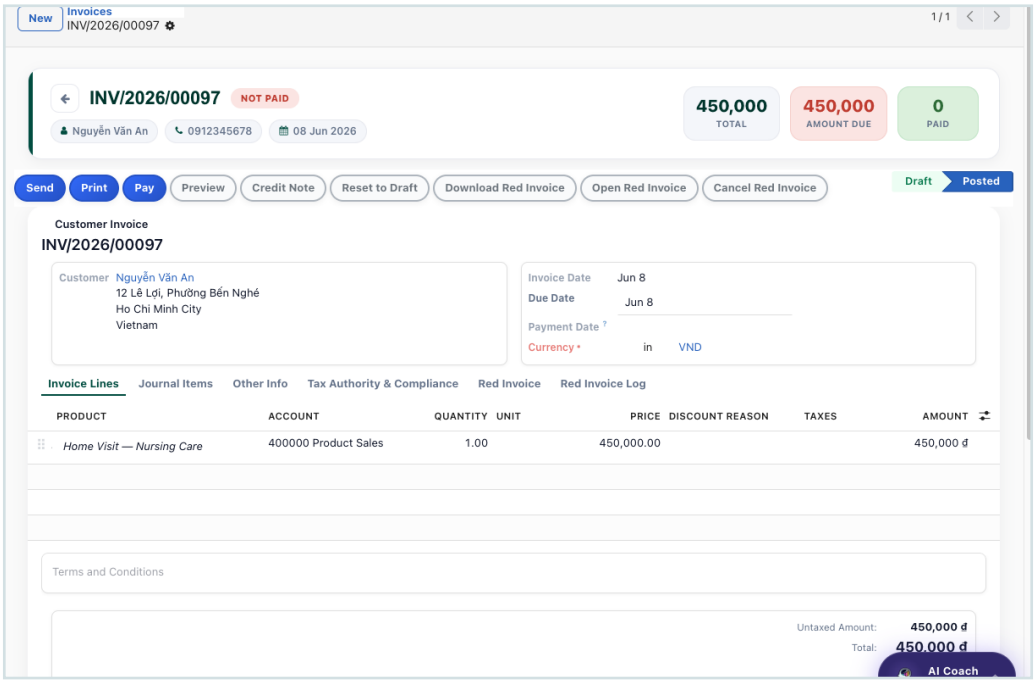


Figure 18. Invoice detail: review document state, lines, total and payment status.

1. Open the booking and look for its existing invoice first. Completion may already have generated one.
2. If invoice creation is required and permitted, use Create Invoice from the relevant booking or quote. Verify the correct client and source reference.
3. Review service quantities, unit prices, discounts, travel or time charges, tax information and total. Compare with the final services actually recorded.
4. Correct an editable draft through the appropriate process. If a posted document is wrong, use the approved credit/correction route instead of changing history.
5. Confirm or Post only after review. Check the resulting invoice state and outstanding balance.
6. Provide the permitted invoice or receipt to the correct recipient. Keep the internal invoice reference for later payment matching.

Do not generate a second invoice because the first is unpaid or because an e-invoice request is pending. Those issues belong to payment follow-up or e-invoice processing.

8.3 Receive a payment or part payment

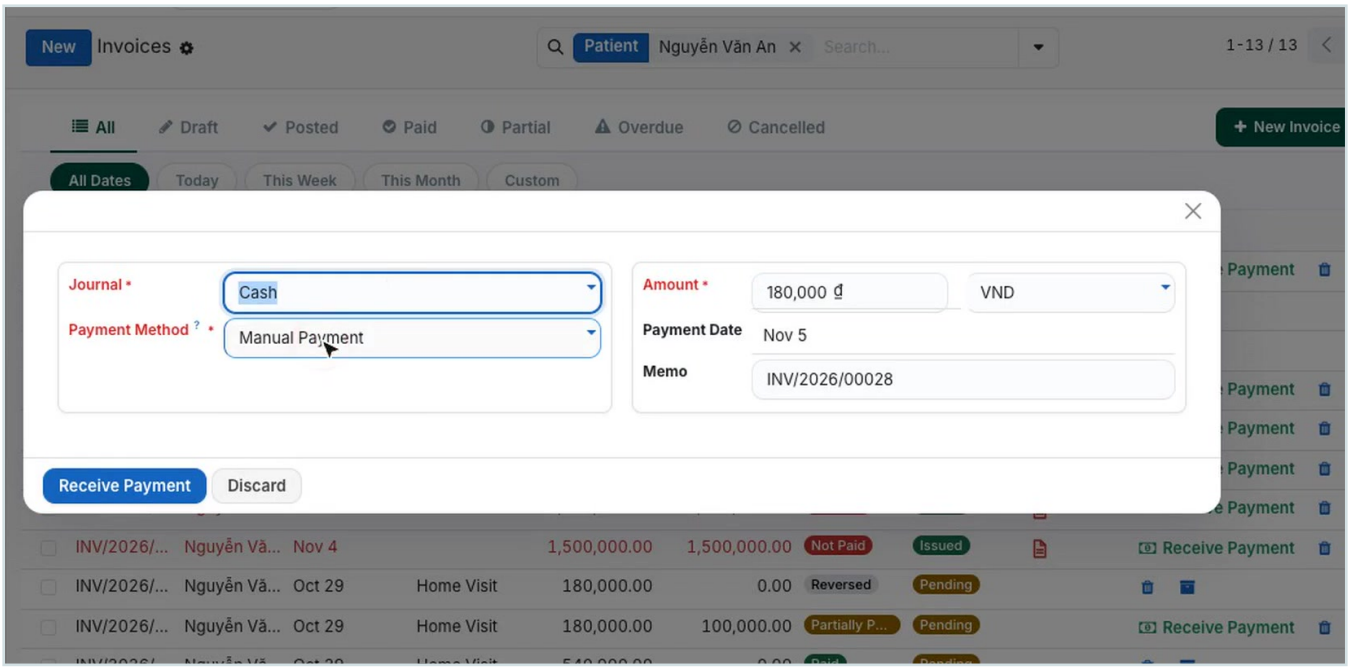


Figure 19. Payment workflow: choose the correct invoice and record the actual amount.

Use Finance > Payments > Account Payment or the payment action on the relevant invoice, according to your role. Record money only when it has actually been received or verified through the approved process.

1. Confirm the client and invoice reference. Check the amount still due.
2. Enter the actual amount received, date, method and payment reference. Select the appropriate journal or receiving account when requested.
3. Review how the amount is allocated. A smaller payment should leave the correct balance; it should not make the whole invoice appear paid.
4. Confirm the action, then reopen the invoice and payment record. Check payment status, amount due and any reconciliation state.
5. For bank transfer, match the bank evidence and reference before recording settlement. For cash, identify who received and holds the money.

Example: an invoice is 1,000,000 VND and the client pays 400,000 VND. The remaining amount is 600,000 VND before any other approved adjustments. Record another actual payment later to settle that remaining amount; do not replace the first payment with the full invoice total.

8.4 Field cash, office receipt and reconciliation

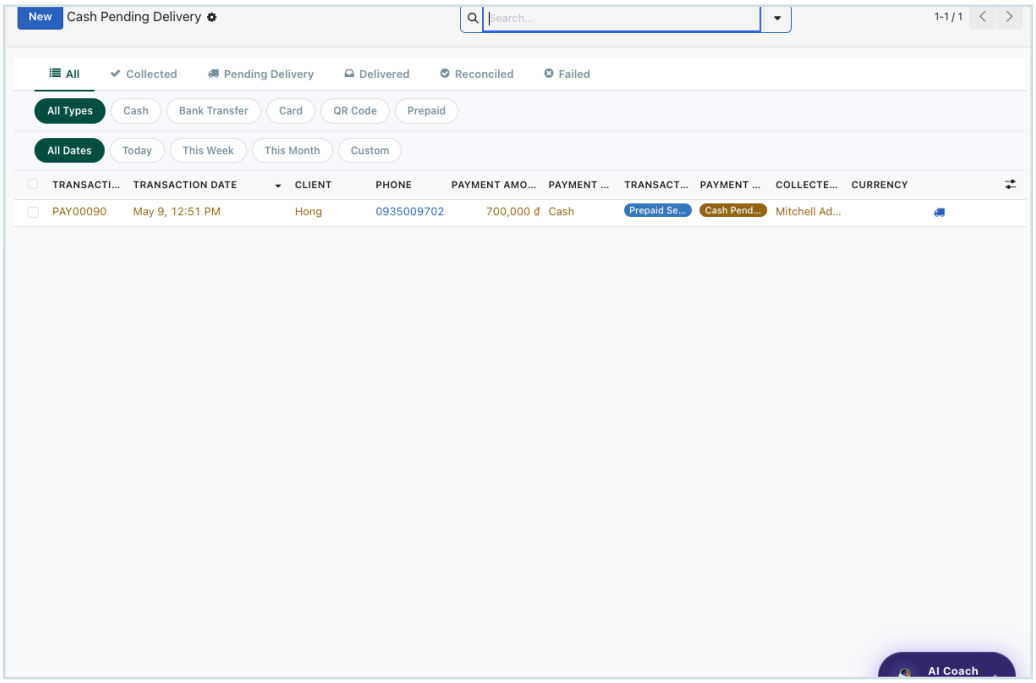


Figure 20. Collections: review staff-held cash and the handover state.

Cash In Transit and Operations > Collections show money collected by staff that still needs office handover. This is different from money the client still owes.

1. The collector records the actual cash against the correct client or invoice through the permitted collection action.
2. At handover, compare the listed payments and references with the cash physically received.
3. Record the receiving/handover action only for the amount verified. Investigate differences rather than silently changing the invoice.
4. Reopen the collection to confirm the holder or delivery state has changed. Finance completes any remaining reconciliation.

Pending Delivery means field cash has not yet completed the office handover. Delivered and Reconciled represent later steps where those statuses are used. Receiving field cash is not a new payment by the client: recording it twice would overstate money received.

At shift end, check outstanding collections by staff member and date, and document who will resolve any discrepancy.

8.5 Receivables, refunds, credit and packages

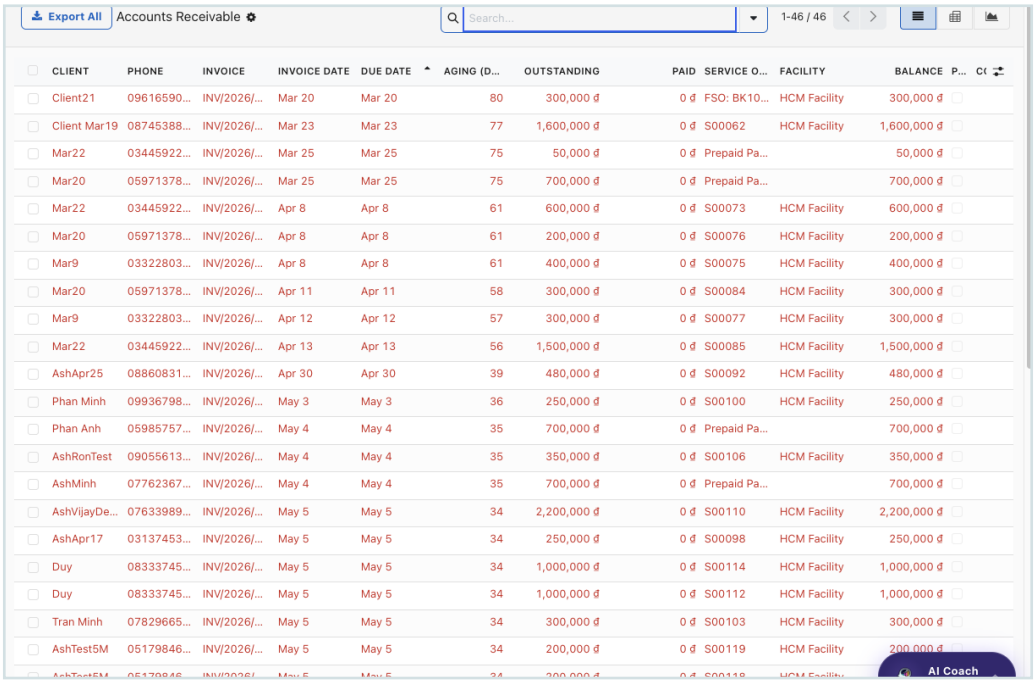


Figure 21. Receivables: inspect the client and remaining amount.

Receivables > AR Dashboard summarises money owed. AR Transactions shows the detailed history. Overdue Clients focuses on unpaid balances past the due date. Filter by client and date, inspect the invoice and payment allocation, then arrange the appropriate follow-up.

A credit note reduces or corrects an invoice through the approved accounting process. A cash refund records money returned. They are related but are not the same event. Use Refund / Credit with the correct original invoice, reason and amount, and verify both the corrected balance and actual money movement.

Service Packages represents prepaid service entitlements. Before selling or applying a package, check the client, included services, validity and remaining entitlement. Link usage to the right visit and inspect the remaining balance afterward. Buying a package is not the same as completing all of its visits. Do not apply an unrelated package just because it has unused value.

For finance exports, use the authorised export option and check the date range, company, record count and totals. An exported file for MISA or another system is not proof the destination has imported and reconciled it.

8.6 VAT log, e-invoices and BHYT claims

VAT Log and Red Invoice Log help follow electronic-invoice processing. Read the provider reference, request status and error details. A local posted invoice is not proof that an external e-invoice was issued. If the request failed, investigate its cause and previous outcome before retrying; avoid duplicate issuance.

BHYT Claims prepares a structured claim from an eligible completed visit and posted invoice. Review the client/card information, invoice link, coverage snapshot, line-by-line covered amounts and patient share. Mark Ready records readiness in this module; it is not confirmation that a national authority has received or accepted the claim. The implemented claim preparation does not itself provide the later national XML/submission workflow.

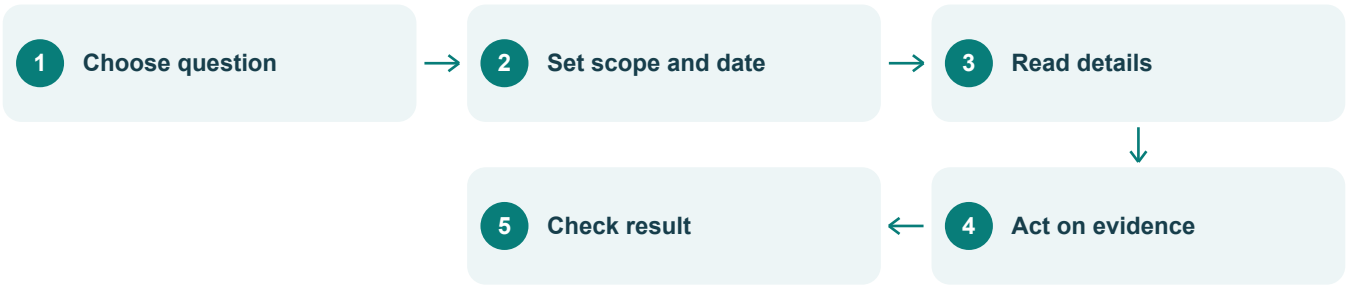
1. Open the correct source document and verify the client's details.
2. Review amounts and any validation issues with authorised finance staff.
3. Use only the available approved workflow and inspect the resulting state.
4. Keep provider or submission evidence distinct from local preparation status.

Ask the responsible finance or integration administrator about a missing connection. Do not change clinical records or invent provider references to clear a financial validation error.

CHAPTER 09

Reports, learning and everyday problem solving

The normal process



In this chapter

Analytics and useful reports

Learn with guides, videos and the coach

Training journeys and practice

When something does not work

Plain-language glossary and daily checklist

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

9.1 Analytics and useful reports

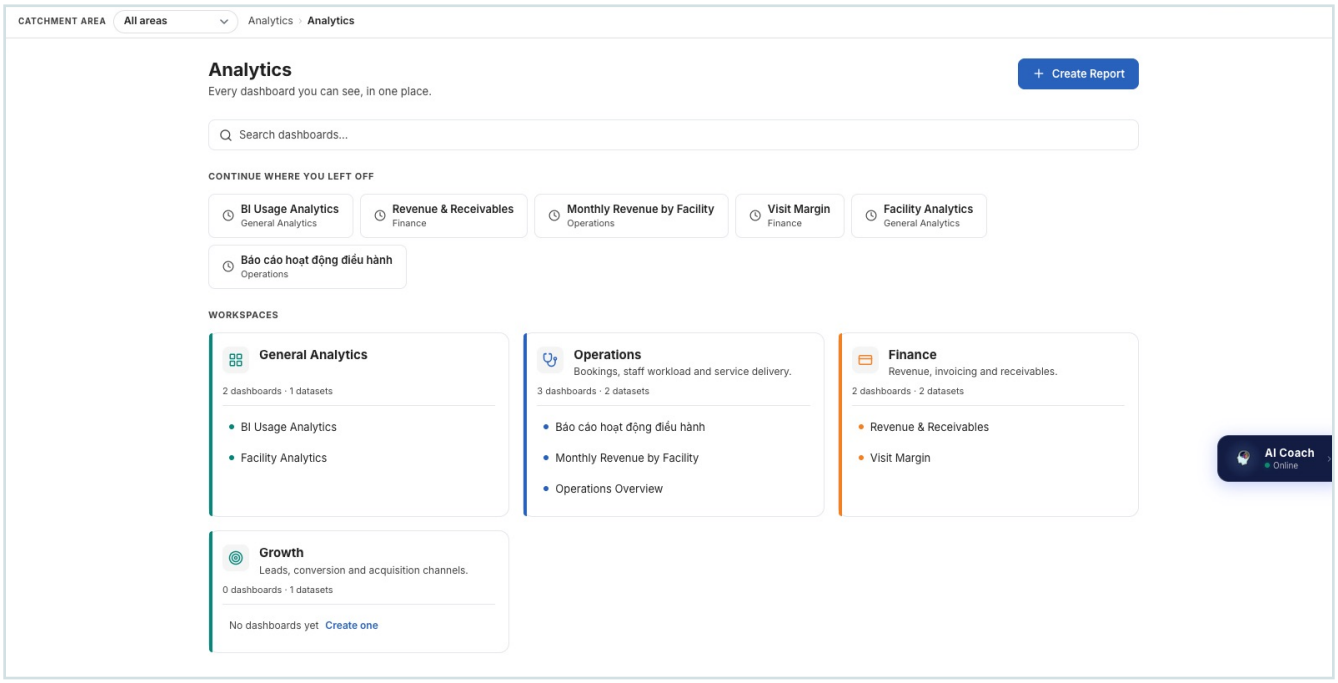


Figure 22. Analytics entry: select the appropriate analysis or dashboard.

Analytics provides permitted dashboards and an Explore workspace. A dashboard answers a defined business question; its figures depend on the selected dates, company, catchment and dataset refresh.

1. Open Analytics and choose the available dashboard relevant to your work.
2. Read the title, measure and filters. Check whether the date refers to enquiry creation, scheduled service, invoice date or payment date.
3. Use Explore to select available data and permitted measures, grouping or filters. Compare a small known set of records before relying on a new analysis.
4. Check totals against the underlying record list where a drill-down is available. A graph shows a summary, not necessarily the same definition as another screen's card.
5. Export or share only through the permitted controls and with the intended audience. Scheduled dashboards use the configured schedule and recipients.

If the data looks old, check the refresh information or ask the reporting administrator. Do not edit source bookings just to make a chart agree. A local report or preview, including an available MOH report, is not evidence of external submission or approval.

9.2 Learn with guides, videos and the coach

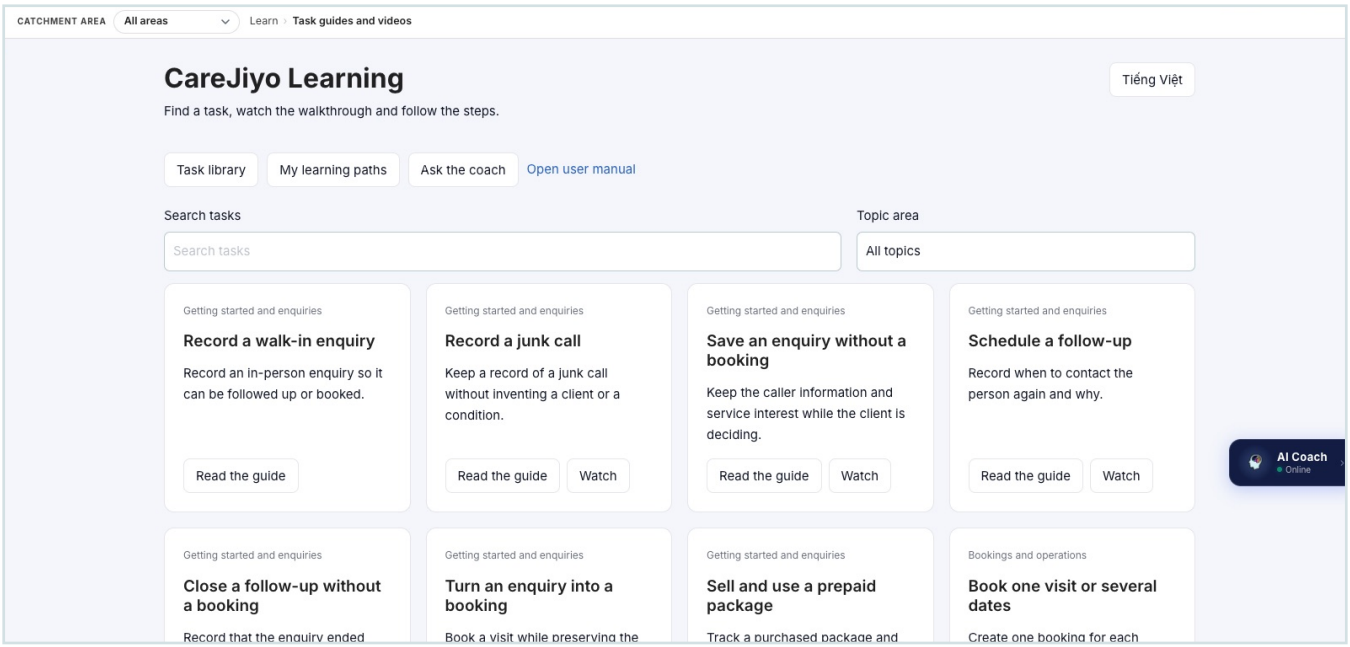


Figure 23. Learning library: search a task, read its guide or watch the video.

Open Learn > Task guides and videos. Search by the task you want to do, such as a follow-up, assignment, clinical note or part payment. Use a topic filter to narrow the results.

Read the goal and Before you begin, then follow the steps. Watch opens the walkthrough; chapter buttons jump to a particular action. Choose English or Vietnamese captions in the player. Reviewed and watched progress belongs to your own account.

My learning paths groups tasks by role. Work through a path, try its knowledge check and read the feedback. A wrong answer can be corrected and tried again. A recorded completion is learning progress, not a professional qualification or permission to perform a restricted action.

Ask the coach uses the published guides to find instructions and link to the relevant topic or video chapter. It does not carry out the task, inspect the current patient's circumstances or give a clinical decision. If it cannot match the question, use search or ask a colleague.

Care Coach > Guides and videos opens relevant help beside a backend screen. In the mobile app, Help opens instructions without leaving the current task. Use these shortcuts when a button or next step is unfamiliar.

9.3 Training journeys and practice

Learn > Training provides the separate training journey where your clinic uses lessons and stations. Follow the lesson instructions, complete the permitted activity and review feedback. Training administrators manage the published lessons and monitor the available learning records.

For practical exercises, use the training records and account approved by your manager. Check the environment before creating a booking, payment, message or clinical record. A practice action in the live system can still create real data or send a real message.

A useful first-day route is: sign in and find a record; read a contact and booking; review your role's workflow; watch one matching video; complete one supervised practice task; explain the result to the trainer. Repeat the workflow until you can recognise both success and an exception.

If instructions and your screen differ, record the screen title, role and button label and ask for clarification. Do not improvise by using a more powerful account.

9.4 When something does not work

What you see	What to check first
No record found	Search text, date field, catchment, company and assignment.
Button missing	Current status and your role; the action may not apply yet.
Required-field error	Read the message, return to the named field and enter accurate information.
Price looks wrong	Service, quantity, date/time, location, catchment, duration and approved discount.
Cannot complete visit	Start state, assignment, notes and required closing answers.
Payment not reflected	Correct invoice, actual saved payment and allocation/reconciliation.
Video will not play	Connection, signed-in account and role; written guide remains the first fallback.
Notification missing	Device permission and actual work list; ask Operations to verify assignment.
Page times out after Save	Reopen/search for the record before repeating the action.

When reporting a problem, provide the screen, record reference, time, expected result and exact message. Describe what you already tried. Send only the information needed to the authorised support route. A screenshot should avoid passwords and unrelated client information.

Do not repeatedly create records, reset prices or change permissions to make an error disappear. Keep the original reference so the responsible team can trace what happened.

9.5 Plain-language glossary and daily checklist

Term	Plain meaning
Catchment	The geographic area assigned to a clinic or staff member.
Lead	An enquiry that may become a booking.
Assignment	The link between a staff member and a visit.
Quote	The proposed charge for the service.
Invoice	The bill for the service.
AR / Receivable	Money still owed to the clinic.
Reconciliation	Matching a payment with the correct financial record.
Consent	Recorded permission for a specified purpose.
eMAR	Electronic medication administration record.
EVV	Electronic record of visit verification events.
Addendum	An additional correction or explanation attached to an existing final record.

Before work: check identity, area, date and your assigned queue. During work: confirm the client, read the details, perform the authorised action, then verify the saved result. Before leaving: review unfinished follow-ups, notes, cash and handovers. Sign out of a shared device after confirming work is saved.

When in doubt, stop at the point you do not understand and use the relevant guide or ask the responsible colleague. Accurate small steps are better than a fast sequence of unverified clicks.